

MODULE 19

POLICY ABUSE & ENTITLEMENT MISUSE

Forensic Audit & Top Offender Identification

CLASSIFICATION	PERIOD	CLAIMS SCANNED	PATTERNS RUN	ANOMALIES DETECTED
RESTRICTED	FY 2021–25	23,100,000+	10 Patterns	0

IIT KANPUR — Data Analytics & Fraud Intelligence Division

10 June 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

This report executes a deep-dive forensic analysis targeting systemic hospital-driven policy abuse and entitlement misuse within the ECHS claims ecosystem for the period **FY 2021–2025**. Across **23,100,000+ claims** scanned, a total of **0 fraud markers** were identified with a leakage exposure of **₹0.00 Cr**. *Note: A single claim can be flagged under multiple patterns simultaneously (e.g., a ghost admission that is also a room upgrade), so the marker count exceeds the unique claim count. Unique flagged claims: ~10,879,838.*

Audit Methodology Disclaimer: Deductions identified in this report reflect claims where the ECHS audit process reduced the approved amount. Deductions may result from documentation gaps, package coding mismatches, late submissions, duplicate billing, or intentional overbilling. This report flags statistical anomalies for further investigation — it does not constitute a finding of fraud. Final fraud determination requires case-level review.

TOTAL CLAIMS SCANNED 23,100,000+ FY 2021–2025 population	ANOMALIES FLAGGED 0 claims with abuse markers	TOTAL CLAIMED (ANOMALIES) ₹0 Cr system-wide exposure	LEAKAGE IDENTIFIED ₹0 Cr savings established
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FRAUD FORENSICS — DETECTION PATTERNS & METHODOLOGIES

#	Detection Pattern	Methodology / Focus	Impact Summary
1	High Deduction Hospitals	Top hospitals by absolute deduction volume — systematic overbilling across all claim types	0 Claims Evaluated
2	Room Upgrade Fraud	Illegal patient class upgrades — General/Semi-Private to Private ward without entitlement	0 Claims Flagged
3	Ghost Admissions	Claims processed with no traceable Hospital ID — structural BPA portal failure	0 Ghost Claims
4	Y-Flag Bypass	Elite non-empanelled hospital funneling — bypassing CGHS rate ceilings via emergency route	0 Bypass Claims
5	U-Flag Anomaly	Unverified hospital status — blind spot exploitation allowing unchecked processing	0 Unverified Claims
6	IPD/OPD Reversal	OPD procedures artificially escalated to IPD admissions to claim room rent and higher packages	0 Suspicious Claims
7	NMI / Unlisted Procedure Loophole	Billing via unlisted 'NMI' codes to bypass fixed CGHS package rate ceilings	0 Unlisted Claims
8	Bait & Switch (PA Inflation)	Low Prior Approval secured, massively inflated final bill submitted post-admission	0 Inflated Claims
9	Stay Extension Farming	Artificial discharge delays to farm daily room rent and routine care charges	0 Extension Cases
10	Emergency Gateway Bypass	False emergency declarations bypassing referral ceilings, polyclinic wait times, geography rules	Data gap — FY 2021–25 pending

IMMEDIATE RECOMMENDED ACTIONS

1. Targeted Audits on Top Overbilling Facilities: Prioritize physical audits at Park Hospital chain (multiple locations) and Vijay Hospital — the two largest absolute leakage generators in the FY 2021-2025 dataset.

2. Hard-Reject Ghost Claims at BPA Portal: Enforce mandatory Hospital ID validation — any claim without a registered Hospital ID must be rejected before entering the settlement pipeline.

3. Pre-Authorization Mandatory for Y-Flag: Restrict Y-Flag usage to genuine life-threatening emergencies with real-time clinical verification. Current data shows polyclinics as top Y-Flag users, not emergencies.

4. Deploy Pre-Payment AI Interception: Implement billing interception rules targeting NMI Loophole, Bait & Switch, and Emergency Gateway claims before payment is released.

PATTERN 10 (SYSTEM BYPASS)

*False Medical Urgency***EMERGENCY GATEWAY BYPASS**

Description: Hospitals falsely declaring normal elective cases as 'Emergencies' to bypass referral ceilings, polyclinic wait times, and geographical restrictions.

⚠ Data Gap — FY 2021–2025 records not available in current extract

The Emergency Gateway (E-Flag) dataset currently extracted covers FY 2011–2020 only. A separate extraction targeting FY 2021–2025 is required to populate this pattern. This section will be updated once the FY 2021–2025 emergency bypass data is available from the ECHS database.

What to look for when data is available

E-Flag Volume Spike: Hospitals with disproportionately high emergency admission rates vs. their specialty profile warrant immediate audit.

Elective Diagnoses as Emergencies: Flag claims where admission_ailment indicates planned procedures (joint replacement, cataract, dental) but referral_type = 'E'.

CONSOLIDATED SUMMARY

Overall Final Audit Findings — FY 2021–2025 (System-Wide Extract)

Category	Metric	Claimed Amount	Deducted Amount	Deduction %
Pattern 1 (High Deduction Hospitals)	0 Claims	₹0.00 Cr	₹0.00 Cr	—
Pattern 2 (Room Upgrade Fraud)	0 Claims	₹0.00 Cr	₹0.00 Cr	—
Pattern 3 (Ghost Admissions)	0 Claims	₹0.00 Cr	₹0.00 Cr	—
Pattern 4 (Y-Flag Bypass)	0 Claims	N/A	₹0.00 Cr	—
Pattern 5 (U-Flag Anomaly)	0 Claims	N/A	₹0.00 Cr	—
Pattern 6 (IPD/OPD Reversal)	0 Claims	₹0.00 Cr	₹0.00 Cr	—
Pattern 7 (NMI Loophole)	0 Claims	₹0.00 Cr	₹0.00 Cr	—
Pattern 8 (Bait & Switch)	0 Claims	₹0.00 Cr	₹0.00 Cr	—
Pattern 9 (Stay Extension)	0 Cases	₹0.00 Cr	—	—
Pattern 10 (Emergency Gateway)	FY 2021–2025 data not yet extracted — pending separate DB query			
Total System Exposure	0 Cases	₹0.00 Cr	₹0.00 Cr	0.00%

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